



MOMETASONE 0.1% CREAM

COMPOSITION:

Mometasone Furoate (micronised) 0.1% w/w
Methylparaben 0.1%w/w as preservative.

PRESENTATION:

White to off white coloured, water miscible cream.

INDICATIONS:

Indicated for the relief of the inflammatory and pruritic manifestations of corticosteroid responsive dermatoses, such as psoriasis and atopic dermatitis.

PHARMACOLOGY:

Mechanism of Action:

Mometasone Furoate, a synthetic corticosteroid, exhibits anti-inflammatory, antipruritic and vasoconstrictive properties.

Pharmacokinetics:

Systemic absorption following topical application of mometasone furoate cream 0.1% is minimal, approximately 0.4% of the applied dose. Mometasone Furoate is extensively metabolized in the liver after topical application. Following percutaneous absorption, topical corticosteroids are excreted primarily by the kidneys and to a small extent in the bile. The elimination half-life is 5.8 hours.

DIRECTION FOR USE:

Apply a thin film of cream to the affected skin areas once daily until the lesion heals or for a duration of three weeks, whichever is sooner.

CONTRAINDICATION:

Contraindicated in patients who are sensitive to mometasone furoate, to other corticosteroids or to any component of this preparation.

PRECAUTION:

If irritation or sensitization develops with the use of Axcel Mometasone 0.1% Cream, treatment should be discontinued and appropriate therapy instituted. In the presence of an infection, use of an appropriate antifungal or antibacterial agent should be instituted. If a favorable response does not occur promptly, the corticosteroid should be discontinued until the infection is controlled adequately. Any of the side effects that have been reported following systemic use of corticosteroids, including adrenal suppression, may also occur with topical corticosteroids, especially in infants and children. Systemic absorption of topical corticosteroids will be increased if extensive body surface areas are treated or if the occlusive technique is used. Suitable precautions should be taken under these conditions or when long-term use is anticipated, particularly in infants and children. Pediatric patients may demonstrate greater susceptibility to topical corticosteroid - induced hypothalamic - pituitary axis suppression and Cushing's syndrome than mature patients because of a larger skin surface area to body weight ratio. Use of topical corticosteroids in children should be limited to the least amount compatible with an effective therapeutic regimen. Chronic corticosteroid therapy may interfere with growth and development of children. This product is not for ophthalmic use.

Use in Pregnancy and Lactation:

Since the safe use of the product in pregnant women has not been established, topical corticosteroids should be used during pregnancy only if the potential benefit justifies potential risk to the fetus. Drug of this class should not be used on pregnant patients in

large amounts or for prolonged periods of time. It is not known whether topical administration of corticosteroids could result in sufficient systemic absorption to produce detectable quantities in breast milk. Systemically administered corticosteroids are secreted into breast milk in quantities not likely to have deleterious effect on the infant. Nevertheless, a decision should be made whether to discontinue nursing or to discontinue the drug, taking into account the importance of the drug to the mother.

Further information can be obtained from pharmacist, physician or the manufacturer.

SIDE EFFECTS:

Local adverse reactions rarely reported include paraesthesia, pruritus and sign of skin atrophy. The following local adverse reactions have been reported infrequently with the use of other topical corticosteroids : irritation, hypertrichosis, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, striae and miliaria.

DRUG INTERACTIONS:

Not known.

OVERDOSAGE AND TREATMENT:

Excessive, prolonged use of topical corticosteroids can suppress pituitary-adrenal function resulting in secondary adrenal insufficiency. Appropriate symptomatic treatment is indicated. Acute hypercorticism symptoms are virtually reversible. Treat electrolyte imbalance, if necessary. In cases of chronic toxicity, slow withdrawal of corticosteroids is advised.

STORAGE:

Keep container well closed. Store below 30°C. Protect from light.

**KEEP OUT OF REACH OF CHILDREN
JAUHI DARI KANAK-KANAK**

PACK QUANTITIES:

Available in aluminium tube of 5gm and 15gm.

Manufacturer & Product Registration Holder :



Kotra Pharma (M) Sdn. Bhd.

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